Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0015	**Age / Sex**	35 / M
Setting	Emergency Dept	**Primary dx**	Alcohol intoxication, minor head injury

Chief complaint

Found asleep on the street with a scalp laceration.

History of present illness

Brought by ambulance after passer-by called. Strong smell of alcohol, unable to give a reliable history. Friends later report heavy drinking over 6 hours and a fall from a bar stool. No vomiting, no seizure.

Past history / medications

Daily alcohol use, no known medical conditions.

Examination

HR 90, BP 130/80, RR 14, temp 35.9 C, glucose 5.1 mmol/L. 3 cm scalp laceration over the right occiput, no skull step-off. Pupils equal, reactive. No lateralizing signs. Slurred speech, ataxic when stimulated.

Glasgow Coma Scale

Eyes open to voice. Verbal output confused and slurred. Obeys commands.

GCS on arrival	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	4	4 - Confused
Motor (M)	6	Obeys commands
TOTAL GCS		**13**

GCS at 6 h	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	5	5 - Oriented
Motor (M)	6	Obeys commands
TOTAL GCS		**15**

Investigations

Blood alcohol 3.1 g/L. CT head (because of reduced GCS and unreliable history): no fracture or hemorrhage. Electrolytes normal. CK normal.

Assessment

Acute alcohol intoxication with minor head injury, GCS 13 on arrival. Reduced consciousness explained by intoxication, intracranial injury excluded.

Plan / disposition

Laceration closed with staples, thiamine IV, observation until sober. GCS 15 at 6 h. Discharged with head-injury advice and addiction service referral.